



Human Rights in the Health Care Setting

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Objective:



- Define Human Rights
- Understand the difference between stigma and discrimination
- Understand the different types of stigma and the drivers of Discrimination
- Identify strategies for reducing Stigma and discrimination in the health care setting.

What are Human Rights?



- Human rights are the fundamental freedoms and entitlements that belong to every person, regardless of nationality, ethnicity, gender, religion, or any other status.
- They are inherent, universal, and inalienable, meaning they cannot be taken away or given up.
- These rights are essential for human dignity, equality, and freedom.

Human Rights Principle Applicable to the Health Care Setting

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- The Right to privacy
 - The right to freedom of expression
 - The right to freedom from discrimination on the ground of (i) being male or female (ii) race, place of origin, class, colour, religion, political opinions
 - The right to enjoy a Healthy environment

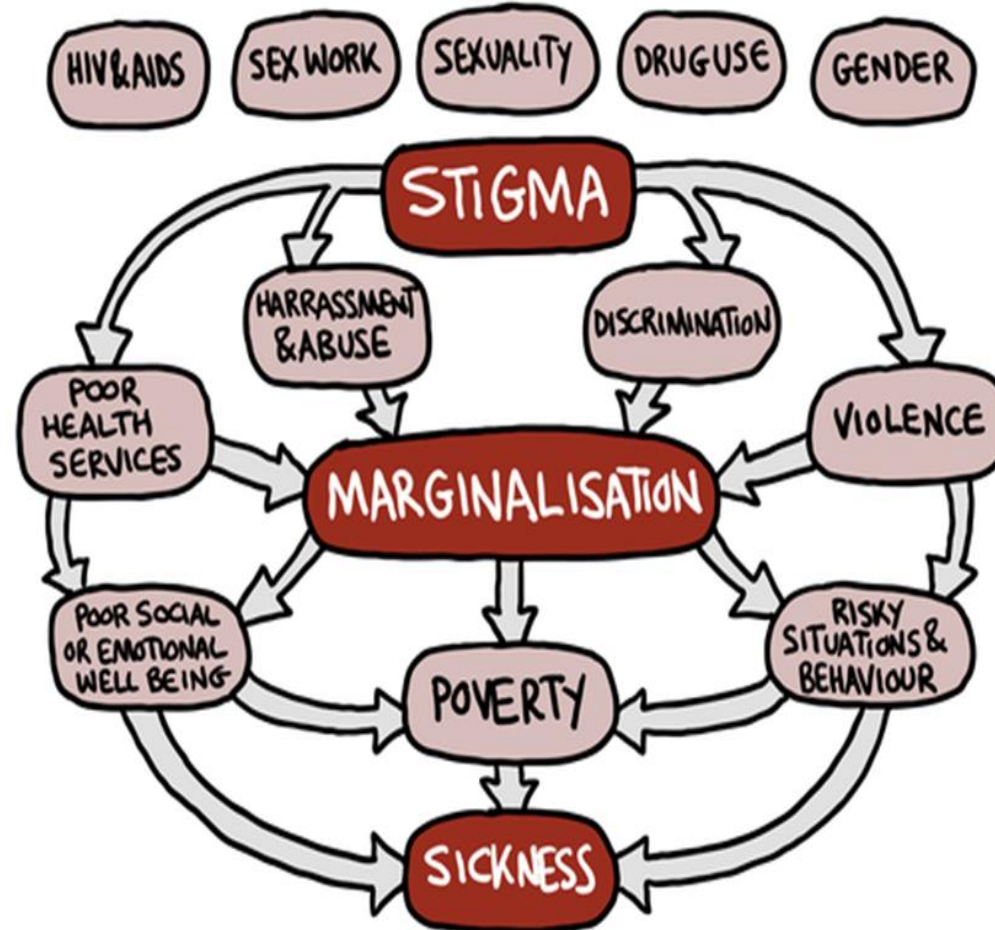


Why are Human Right Important

- Human rights are essential for ensuring dignity, equality, and freedom for all individuals.
- They protect people from discrimination and abuse, ensuring that everyone has access to basic needs, including health care, education, and fair treatment.
- In the health care setting, human rights promote patient-centered care, respect for privacy, and equitable access to medical services.

What is Stigma?

“A social process of devaluing a person , beginning with marking or labeling someone’s differences, then attributing negative values to those differences”



Types of Stigma

Personal Stigma – A person's stigmatizing attitudes and beliefs about others.

Eg: A healthcare provider assuming that a person with a certain condition is irresponsible or careless.

Self-Stigma – The stigmatizing views that individuals hold about themselves.

Example: A person living with HIV feeling ashamed and avoiding seeking medical care due to fear of judgment.

Structural Stigma – The policies of private and governmental institutions that restrict the opportunities of people.

Example: A hospital implementing policies that make it difficult for transgender individuals to access appropriate medical services.

CYCLE OF IGNORANCE



Explanation of Cycle



To visualize these concepts, envision the cycle of ignorance.

Stigma can be part of a vicious cycle where fear, inaction, and silence lead to ongoing stigma and discrimination.

Within a health setting delivering HIV services, stigma and discrimination are among many factors that contribute to disengagement from care and worse health outcomes in an era of potent HIV prevention and treatment options.

What is Discrimination?



- “**Unfair** and **unjust** treatment of an individual based on a **real** or **perceived** characteristic”
- Discrimination can be experienced at individual, facility, community, or national levels

Discrimination can be experienced at:



Individual Level – When a person is treated unfairly by another individual due to their characteristics (e.g., a doctor refusing to treat a patient based on their identity).

Facility Level – When discrimination occurs within a healthcare facility, such as policies or practices that limit access to services for certain groups.

Community Level – When societal norms or cultural biases contribute to exclusion or mistreatment of certain populations.

National Level – When government policies or laws create barriers to equal healthcare access for marginalized groups.

Stigma Index 2.0

	HIV Care		Non HIV Care	
	ALL (N=557)		All (N=190)	
	N	%	N	%
At least one experience of stigma or discrimination in a health care setting	54	10%	29	15%



The diagram presents data from the **Stigma Index 2.0**, comparing experiences of stigma or discrimination in healthcare settings between individuals receiving **HIV care** and those receiving **non-HIV care**.

Key Findings:

1. HIV Care Group (N=557):

1.54 individuals (10%) reported experiencing at least one instance of stigma or discrimination in a healthcare setting.

2. Non-HIV Care Group (N=190):

1.29 individuals (15%) reported experiencing at least one instance of stigma or discrimination in a healthcare setting.

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- Surprisingly, the percentage of individuals in non-HIV care (15%) who experienced stigma is higher than those in HIV care (10%).
 - This suggests that stigma in healthcare settings is not exclusive to people receiving HIV care but may also affect those in other healthcare contexts.

Community Compass 2023

87.8% of PLHIV thought that the health facility keeps their HIV status Private and Confidential.

This indicates a strong level of trust in the healthcare system regarding confidentiality.

93% of recipients of care have not experienced stigma and discrimination from anyone in their health facility.

This suggests that most healthcare settings provide an environment free from overt discrimination.

There were no reports of people being refused services due to discrimination based on their HIV Status.

This implies that PLHIV are receiving the care they need without being turned away.

The image presents key findings from a Community Compass survey related to stigma, confidentiality, and access to healthcare for people living with HIV (PLHIV). The three main points are:

- **Confidentiality of HIV Status**

87.8% of PLHIV believe that health facilities keep their HIV status private and confidential.

This indicates a strong level of trust in the healthcare system regarding confidentiality.

- **Stigma and Discrimination in Healthcare Facilities**

93% of care recipients reported that they have not experienced stigma or discrimination from anyone at their healthcare facility.

This suggests that most healthcare settings provide an environment free from overt discrimination.

- **Overall Interpretation:**

- The data suggests a positive trend in reducing HIV-related stigma in healthcare settings.
- Most PLHIV feel their status is kept confidential, experience little to no discrimination, and do not face service denial due to their HIV status.

STIGMA ↔ DISCRIMINATION

- **Stigma is a belief or attitude**
- **Discrimination is the action resulting from stigma**
 - For example:
 - People living with HIV being refused treatment in a health facility
 - A patient's HIV status or sexual identity being revealed publicly
- **Discrimination takes many forms**
 - Denial of services
 - Physical or verbal abuse
 - Involuntary treatment
 - Forced contraception or abortion



As health workers, we sometimes automatically make judgments about people without realizing how these will affect them or the health services they receive

Source: Health Policy Project

- **Stigma and discrimination are not mutually exclusive.** They are intimately related. While stigma is a belief or attitude, the action resulting from stigma is discrimination.
- **For example,** People living with HIV may be refused treatment in a health facility due to their HIV status or because they belong to a highly stigmatized key population such as people who inject drugs. A patient's HIV status or sexual identity may be revealed publicly in egregious violation of the right to confidentiality.

DRIVERS OF HIV-RELATED DISCRIMINATION



Lack of awareness

Health workers may be unaware that their attitudes, words & actions are stigmatizing



Moral judgement

Health workers may make negative judgments about people who are “different”

May not understand the lives, identities & sexuality of key populations vulnerable to HIV

MSM, transgender individuals, sex workers & PWUD may be seen as sinful or immoral, thus deserving of shame & blame



Fear & ignorance

Health workers may lack knowledge about & have misconceptions about HIV transmission & fear acquiring HIV through casual contact or medical procedures

Such fear & ignorance drives stigma

Drivers of HIV-Related Discrimination

Lack of awareness among health workers is one driver.

Health workers are often unaware that their attitudes, words, and actions are stigmatizing.

Eg Doctors sometimes unconsciously use two gloves when treating patients and not realizing they are being discriminatory.

A second driver is the application of moral judgments.

Health workers may make negative judgments about people who are “different” and may not understand the lives, identities, and sexuality of key populations vulnerable to HIV. People living with HIV, men who have sex with men, transgender individuals, sex workers, and people who use drugs may be viewed as sinful or immoral, thus deserving of shame, blame, and ill treatment.

Fear and Ignorance

Health workers may lack knowledge about and have misconceptions about HIV transmission and therefore fear acquiring HIV through casual contact or medical procedures, such as drawing blood. Such non-evidence-based fear and ignorance drive stigma.

KEY POPULATIONS MOST AT RISK OF DENIAL OF CARE

- Adolescents & young people
 - Particularly adolescent girls & young women
- Persons living with HIV
- Men who have sex with men
- Transgender persons
- Sex workers
- People who use drugs



EFFECTS OF STIGMA ON KEY POPULATIONS



Feelings of shame & self-doubt



Anxiety & depression



Excessive alcohol & drug use



Changing to another health facility, private doctors or self-treatment



Delayed HIV treatment initiation
HIV treatment interruption or stopping treatment altogether



Non-disclosing & taking more risks such as condom-less sex & not using clean needles



Hiding sexual orientation, drug use, or sex work

REPORTING CASES OF HIV-RELATED DISCRIMINATION



JAMAICA ANTI-DISCRIMINATION SYSTEM FOR HIV (JADS)



The Jamaica Anti-Discrimination System for HIV (JADS) is a national system that collects cases of HIV-related discrimination and refers them to the appropriate entities for redress.



The first complaint was documented through JN+ in 2005.



The Enabling Environment and Human Rights Unit of the National Family Planning Board provides financial and technical support to the system



JADS is guided by a Steering Committee consisting of multi-sectoral partners

WHY IS JADS IMPORTANT?

Plays an important role in reducing stigma and discrimination towards people living with HIV, and Gender-based violence.

Advocates for people's human rights

Connects individuals with redress options!



WHY DO CASES NEED TO BE REPORTED?



To empower people living with and affected by HIV



To evaluate laws & institutions
(identify gaps for law revision & institutional changes)



To increase accountability

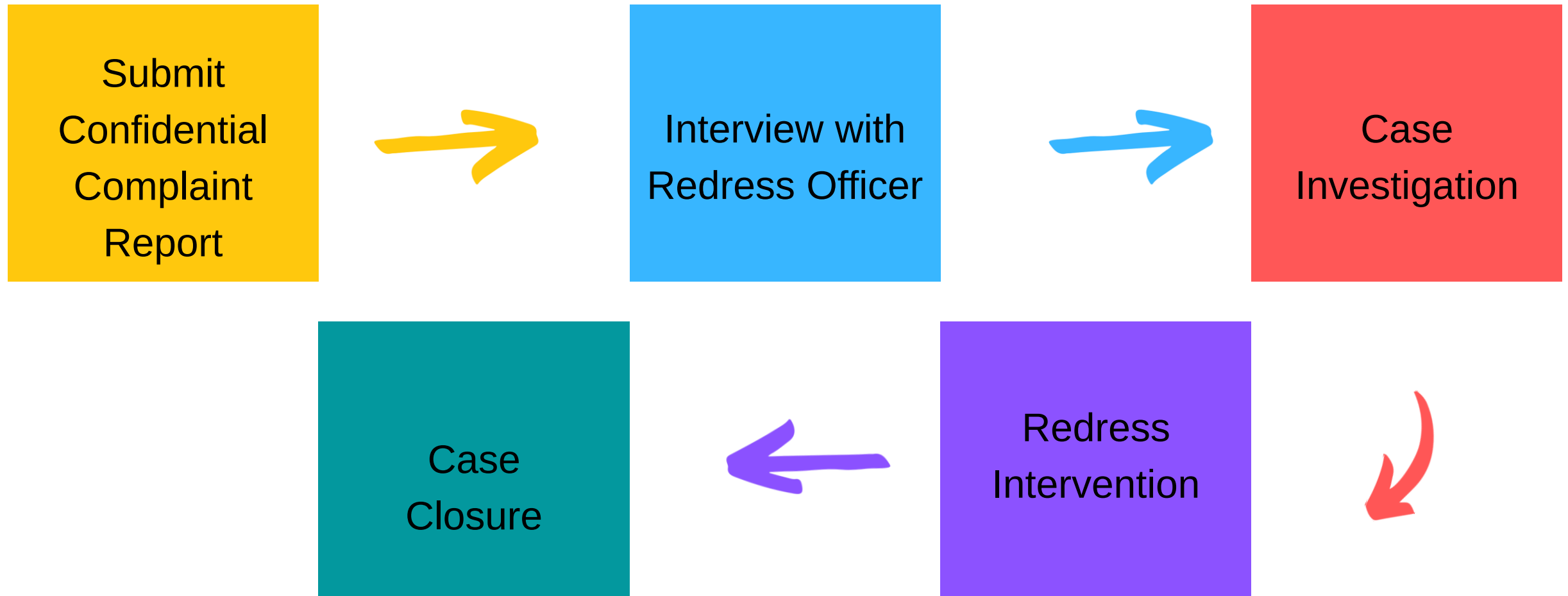


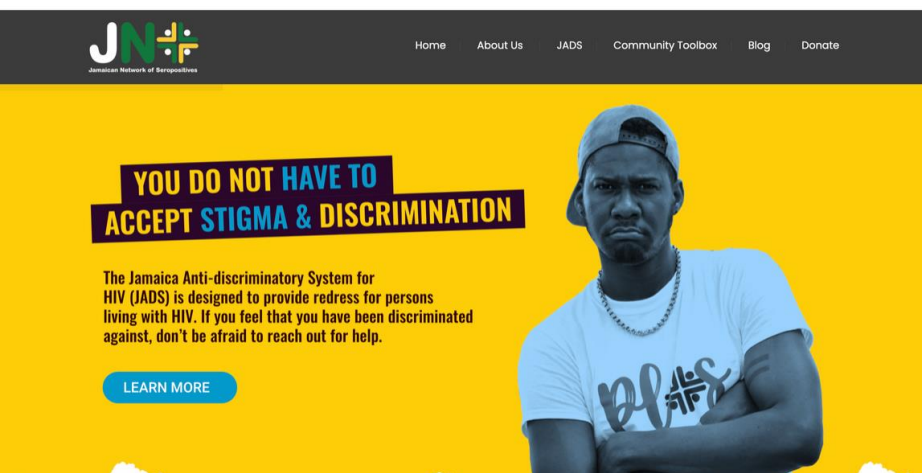
To measure the effectiveness of
strategies & initiatives, and design
new ones

JADS REPORTING FORM



HOW DOES JADS WORK?





SUBMIT COMPLAINT REPORT

1

Who should file the complaint?

1. Persons experiencing discrimination.
2. Witnesses to discrimination, reports, and more.

2

Where can the complaint be reported?

1. JN+ office, online at jnplus.org or by phone at 929-7340/839-8000
2. Jamaica AIDS Support for Life, Jamaica Red Cross, CHARES, J-FLAG, Children First, Client Complaint Mechanism at all Regional Health Authorities
3. Adherence counsellors & social workers at treatment sites

JN+ Community Facilitators within the regions and Retention Navigator within the Southeast and Southern Region.

3

How does JADS ensure confidentiality?

Governed by a confidentiality policy and each incident is assigned a unique code in the database.



INTERVIEW

1

Who conducts the interview?

Redress Officers with in JN+ conduct interview with complainant/person experiencing discrimination

2

What is the purpose of the interview?

1. To provide detailed information about the incident
2. To establish a course of action desired by the complainant for the pursuit of redress.

3

Possible courses of action (dependent on complainant's wishes):

- 1.Receive an Apology
2. Referral for Social Assistance
- 3.Received Counseling
- 4.Conflict Resolution
5. Referral for medical Care
- 6.Legal representation
- 7.Report to Police
- 8.No Action (I just want to file a report)

REVIEW PANEL

Who leads the Review Panel?

1. Review Panel (made up of representatives from entities that have a mandate to provide redress)
2. External authority such as (Ministry of Health Complaint Management System, Ministry of Labour & Social Security, Jamaica Constabulary Force, etc.)

What is the purpose of the Review Panel?

1. To examine detailed information of the case
2. To determine the validity of accusations.

Advancement to next steps depends on:

1. Complainant's wishes
2. Merits of case
3. Redress options available



CLOSURE

Reasons for case closure:

1. Complainant cannot be reached to complete the interview
2. Complainant requests, at any time, to close the case
3. Investigative team recommends case closure
4. Redress intervention is complete.



REPORTING

Non-identifying information from the complaint is shared with:

1. Ministry of Health Monitoring & Evaluation Unit
2. Stakeholders
 - Civil society organizations
 - Government agencies
 - PLHIV community
3. Researchers
 - UWI Repository



JADS DATA

88 cases collected in 2024



Total Cases for the Year 2024	Nature of Incident	Gender	Setting	Parish	Redress Status
88	Breach of confidentiality: 23	Woman:64	Community: 70	St Elizabeth: 7 Manchester: 3	Closed: 69
	Harassed/Verbally Abused: 60	Man:23	Health Facility: 6	Clarendon:3 Westmoreland:14	In review-:18
	Employment: 1	Anonymus:1	School: 2	St James:7 St Ann:2	Anonymous :1
	Unfair Treatment at Hospital: 2	Transgender Female: 0	Workplace: 6	St Mary: 31 St Thomas:1	
	Verbal & Physical Violence: 1	Transgender Man : 0	Social Media: 2	Kingston: 6 & St Andrew: 1 St Catherine: 6	
	Anonymous:1		Police Station: 1	Hanover: 1	
			Anonymous:1	Trelawny:6	
			Other: 1		

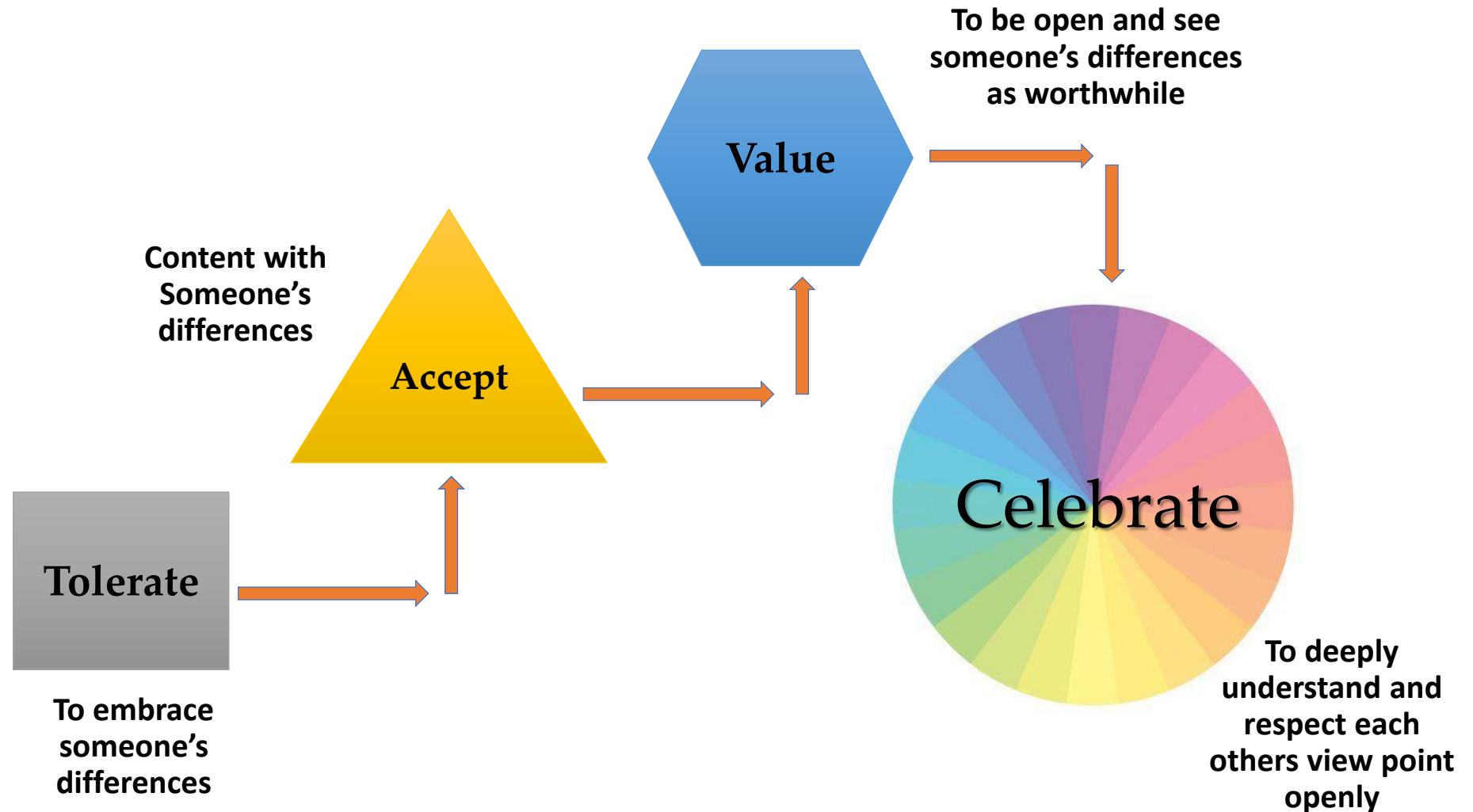


How can we Reduce Health- Related Discrimination?

EMBRACE DIVERSITY EQUITY & INCLUSION

- 
1. Be more concerned with truth than opinions/ Be open to learn
 2. Be sincere and not pretend
 3. Know who you are and to be that person
 4. Seeing your own vulnerabilities
 5. Be awake to your own subconscious biases /feelings
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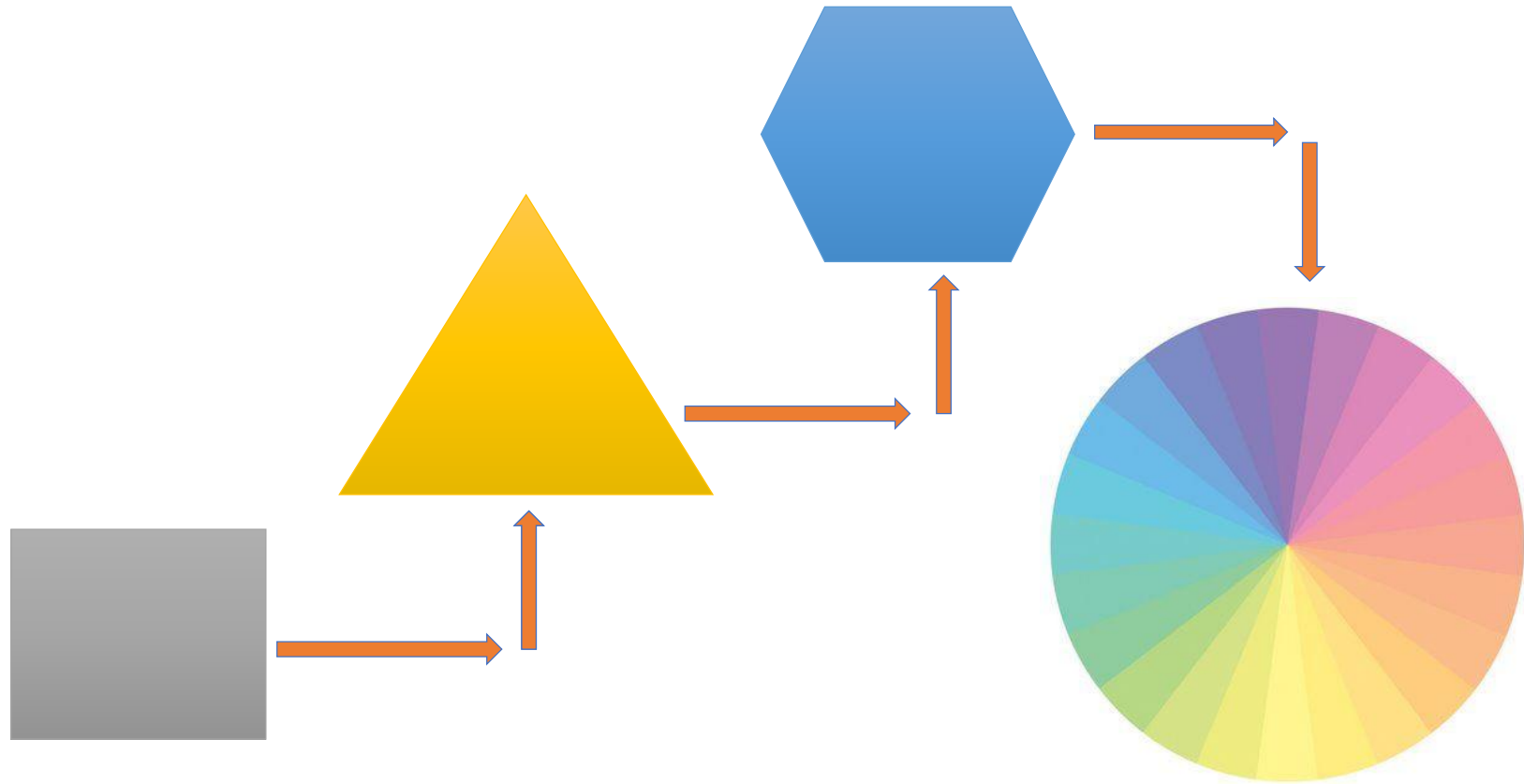
At What Level Do You Value Differences?



The image presents a conceptual framework titled "At What Level Do You Value Differences?", which illustrates a progressive approach to recognizing and appreciating diversity. It features four levels:

1. **Tolerate (Lowest Level)** – At this stage, a person merely puts up with differences but does not necessarily embrace or accept them. It implies minimal effort in understanding others' uniqueness.
2. **Accept** – This level indicates being content with differences. The individual acknowledges them but may not actively seek to engage with or understand them.
3. **Value** – At this stage, a person is open to differences and sees them as worthwhile. This means actively appreciating diversity and recognizing its importance in society.
4. **Celebrate (Highest Level)** – This represents the highest form of valuing differences. It involves deeply understanding, respecting, and openly engaging with others' perspectives in a meaningful way.

Where are YOU Now?





Thank You!

THANK
YOU
FOR
LISTENING

**YOU DO NOT HAVE TO
ACCEPT STIGMA & DISCRIMINATION**

The Jamaica Anti-discrimination System for HIV (JADS) is designed to provide redress for persons living with HIV. If you feel that you have been discriminated against, don't be afraid to reach out for help.

Are you being treated badly because
of your HIV status?
Call JADS It's your right

876-929-7340 or 876-839-8000

